Behavioral Health Agency Inspection Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

September 29, 2022	

Newport Academy, 3804 Hastings Ave W, Port Townsend, WA. 98368-9642	Michael Tyers
Agency Name and Address	Administrator

ONGOING - ROUTINE	09/28/2022	RXE03
Inspection Type	Inspection Onsite Dates	Inspector

X2022-374	BHA.FS.60957239
Inspection Number	License Number

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the on-site inspection.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction

0910 Clinical record content WAC 246-341-0640(1)(a) Each agency is responsible for the components and documentation in an individual's clinical record content unless specified otherwise in specific service certification requirements. (1) The clinical record must include: (a) Documentation the individual received a copy of counselor disclosure requirements as required for the counselor's credential.	The Washington Administrative Code (WAC) is not met as evidenced by: Based on clinical record review, policies, and procedures (P&P) review and staff interview, it was determined the agency failed to ensure each resident received a copy of the counselor disclosure requirements in accordance with MH/A WAC 246-810-031 Disclosure Statement content for 4 of 4 records reviewed. ***Reference:*** *SUDP/T WAC 246-811-90* *A substance use disorder professional and a substance use disorder professional trainee must provide client disclosure information.* *A substance use disorder professional and a substance use disorder professional trainee must provide disclosure information to each client prior to the delivery of certified services. Disclosure information may be printed in a format of the substance use disorder professional's choosing or in a general format used by a state approved treatment facility.* ***Reference:*** *SUDP/T WAC 246-811-100* ***Disclosure statement requirements.*** *(1) The following information must be printed on all disclosure statements provided to counseling clients in language that can be easily understood by the client:* *(a) Name of firm, agency, business, or substance use disorder professional's practice.* *(b) Employment address and telephone number.* *(c) Name, credential, and credential number.* *(d) Billing information, including:* *(i) Client's cost per each counseling session;* *(ii) Billing practices, including any advance payments and refunds.* *(e) A list of the acts of unprofessional*	Counselor Disclosure form included. All clients will sign this upon admission. Program Manager does spot audits to make sure all consents are being signed. Compliance Department completed monthly audits.

0935 Clinical record content
WAC 246-341-0640(1)(c)(iv) Each agency is responsible for the components and documentation in an individual's clinical record content unless specified otherwise in specific service certification requirements. (1) The clinical record must include: (c) An assessment which is an age-appropriate, strengths-based psychosocial assessment that considers current needs and the individual's relevant behavioral and physical health history according to best practices, completed by a person appropriately credentialed or qualified to provide the type of assessment pertaining to the service(s) being sought, which includes: (iv) A diagnostic assessment statement, including sufficient information to determine a diagnosis supported by the current and applicable Diagnostic and Statistical Manual of Mental Disorders (DSM-5) or a placement decision, using ASAM criteria dimensions, when the assessment indicates the individual is in need of substance use disorder services.

Washington Administrative Code is not met as evidence by:

Based on clinical record review, P&P review, and staff interview, it was determined the agency failed to document a **diagnostic assessment statement** supported by. the current and applicable *American Psychiatric Association: Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition. Arlington, VA, American Psychiatric Association, 2013.* (DSM-5).

The agency failed to **document placement decision using ASAM criteria dimensions**, when the assessment indicates the individual is in need of substance use disorder services.

Failure to ensure that a diagnostic assessment statement includes sufficient data to determine a diagnosis supported by the (DSM-5) may result in inaccurate diagnosis as well as an inaccurate level of care recommendation for treatment services.

Failure to ensure the ASAM criteria is used in making placement decisions places residents at risk of not receiving the appropriate level of care for treatment services.

> Reference: *The ASAM Criteria, Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions, American Society of Addiction Medicine, Third Edition, 2013*, Page 110.
> “The ASAM criteria multidimensional assessment helps ensure comprehensive treatment. In the process of patient assessment, certain problems and priorities are identified as justifying admission to a particular level of care. The resolution of those problems and priorities determines when a patient can be treatment at a different level of care or discharged from treatment. The appearance of new problems

Licensing and Compliance Manager is working with Electronic Health Record developers to build specific ASAM Dimensions into the Pre-Admission Assessment (VAIR), this will be filled out if the client is given a primary substance use diagnosis during the clinical clearance process. This should be released for use within the next 30-45 days.

In the meantime, clinicians complete the clinical clearance have been trained to documents ASAM criteria for any client with a primary substance use diagnosis.

In the new form, the user is already prompted to write a clinical case conceptualization summary and symptoms to support DSM-V diagnosis.

Our National Clinical Team team does periodic audits of Clinical Clearances to ensure correct rationale is being given for the level of care the client is admitted to and that there are specifics to support the given diagnosis.

0975 Clinical record content WAC 246-341-0640(1)(g) Each agency is responsible for the components and documentation in an individual's clinical record content unless specified otherwise in specific service certification requirements. (1) The clinical record must include: (g) If treatment is for a substance use disorder, documentation that ASAM criteria was used for admission, continued services, referral, and discharge planning and decisions.	The Washington Administrative Code (WAC) was not met as evidenced by: Based on clinical record review, P&P review, and staff interview, it was determined the agency failed to document in clinical records a review of ASAM dimensional criteria to determine the need for continued services at the same level of care or a review of progress and changing to a lower level of care. The ASAM Criteria, Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions, American Society of Addiction Medicine, Third Edition (ASAM). Reference: Mee-Lee D, Shulman GD, Fishman MH, Gastfirend DR, Miller MM, eds. The ASAM Criteria: Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies©; 2013. Page 110. "The ASAM criteria multidimensional assessment helps ensure comprehensive treatment. In the process of patient assessment, certain problems and priorities are identified as justifying admission to a particular level of care. The resolution of those problems and priorities determines when a patient can be treatment at a different level of care or discharged from treatment. The appearance of new problems may require services that can be effectively provided at the same level of care, or that require a more or less intensive level of care. As the patient's response to treatment is assessed, new priorities for recovery are identified…At such time, changing the level of care or changing the program should be based upon a reassessment of the treatment plan with modifications to achieve a better therapeutic response. In cases where some extenuating circumstances must be considered, further justification and presentation of rationale should	Licensing and Compliance Manager is working with Electronic Health Record developers to build specific ASAM Dimensions into the Pre-Admission Assessment (VAIR), this will be filled out if the client is given a primary substance use diagnosis during the clinical clearance process. This should be released for use within the next 30-45 days. In the meantime, clinicians complete the clinical clearance have been trained to documents ASAM criteria for any client with a primary substance use diagnosis. Clinical Director, Nina Foster will monitor this. Our National Clinical Team team does periodic audits of Clinical Clearances to ensure correct rationale is being given for the level of care the client is admitted to and that there are specifics to support the given diagnosis.

Introduction
We require that you submit a plan of correction for each deficiency listed on the inspection report form. Your plan of correction must be submitted to the DOH within ten business days of receipt of the list of deficiencies.

Descriptive Content
Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates

The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring
Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC
Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies in your inspection report you will be sent a letter detailing why your POC was not accepted.

Questions?
Please review the cited regulation first. If you need clarification, or have questions about deficiencies you must contact the inspector who conducted the onsite inspection, or you may contact the supervisor.

STATE OF WASHINGTON
DEPARTMENT OF HEALTH
PO Box 47874 • Olympia, Washington 98504-7874

November 17, 2022

Crystal Goodwin, Regional License Compliance Manager
Newport Academy
3804 Hastings Ave W
Port Townsend, WA 98368

Dear Ms. Goodwin:

The Washington State Department of Health conducted an inspection at Newport Academy - 3804 Hastings Ave W - Port Townsend, WA 98368

Your inspection was conducted on September 29, 2022. The Plan of Correction that was submitted on November 17, 2022, was approved on November 17, 2022. No further action is required.

I sincerely appreciate your cooperation and hard work during the inspection process and look forward to working with you again in the future.

Sincerely,

Rosa Elvia, SUDP
Behavioral Health Reviewer